

1. Administrative & Study Identification

Full Study Title: A Study of AZD0486 Monotherapy or in Combination With Other Anti-Cancer Agents for Mature B-Cell Malignancies **Official Regulatory Title:** A Phase I/II Open-Label Multi-Centre Master Protocol to Evaluate the Safety and Efficacy of AZD0486 Monotherapy or in Combination With Other Anticancer Agents in Participants With Mature B-Cell Malignancies **Short Title/Acronym:** Soundtrack-E **Protocol Number:** D7407C00001 **NCT Number:** NCT06564038 **Sponsor Name:** AstraZeneca **Investigational Product:** AZD0486 (CD19xCD3 bispecific T-cell engager); Acalabrutinib; R-CHOP **Phase of Development:** Phase I/II **Trial Status:** Recruiting **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety and tolerability of AZD0486 administered as monotherapy and in combination with other anticancer agents across mature B-cell malignancies, and to determine the recommended Phase 2 dose. **Secondary Objectives:** To evaluate preliminary efficacy endpoints (ORR, CR, DoR), pharmacokinetics, and immunogenicity of AZD0486.

2.2 Background & Rationale AZD0486 is an IgG4 fully human CD19xCD3 bispecific T-cell engager that binds CD3 with low affinity, aiming to reduce cytokine release upon T-cell activation while preserving effective cytotoxicity against malignant B cells. This master protocol evaluates the drug in patients with Relapsed/Refractory (R/R) Chronic Lymphocytic Leukaemia (CLL), Small Lymphocytic Lymphoma (SLL), Mantle-cell Lymphoma (MCL), Large B-cell Lymphoma (LBCL), and B-cell Non-Hodgkin Lymphoma (B-NHL) to address unmet needs in advanced hematological malignancies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm/Dose-escalation cohorts **Blinding:** Open-label **Randomization:** N/A (Sequential assignment to cohorts based on substudy)

3.2 Planned Enrollment & Duration Targeted Enrollment: 276 total participants **Number of Sites:** Multi-center (Global) **Estimated Study Start:** 30-Jan-2025 **Estimated Primary Completion:** 28-Feb-2028

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Master Inclusion Criteria: 1. Age ≥ 18 years with an Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 2. 2. Contraception use during treatment and at least 90 days after final dose (for Substudy 3: at least 90 days after last dose of AZD0486 or 4 months after vincristine, and 6 months after cyclophosphamide or doxorubicin). 3. Confirmed CD19 expression if prior anti-CD19 therapy was received. **Substudy 1 (CLL/SLL):** Require treatment per iwCLL criteria (CLL) or have ≥ 1 measurable site per Lugano (SLL); absolute lymphocytes $<10,000$. Cohort 1A/1C require ≥ 2 prior lines of systemic therapy. Cohort 1B requires ≥ 1 prior line and must be BTKi-sensitive. **Substudy 2 (MCL):** MCL diagnosis per WHO; Clinical Stage II, III, or IV by Ann Arbor Classification; ≥ 1 measurable site per Lugano; ALC $<10,000$. Cohorts 2A/2C require relapse or progression after ≥ 2 lines of therapy including BTKi. **Substudy 3 (LBCL/B-NHL):** LBCL per WHO 2022 or R/R B-NHL after ≥ 1 prior line of therapy; International Prognostic Index (IPI) 2-5; ≥ 1 measurable site per Lugano; LVEF $>50\%$.

4.2 Exclusion Criteria Master Exclusion Criteria: 1. Central nervous system (CNS) lymphoma. 2. Surgery within 14 days of study drug or clinically significant cardiovascular (CV) disease. 3. Unresolved Grade >2 AEs from prior anticancer therapy (except alopecia or fatigue). 4. Any anticancer therapy within 5 half-lives or 21 days (whichever is shorter) prior to treatment; radiation therapy within 28 days. 5. Prior CAR T-cell therapy or autologous-haematopoietic stem cell transplant (HSCT) within 12 weeks, or prior T-cell engager (TCE) within 8 weeks. 6. Prior Grade >3 cytokine release syndrome (CRS) or immune effector cell-associated neurotoxicity syndrome (ICANS) event. 7. Prior allogeneic HSCT or solid organ transplantation within 24 weeks of starting Cycle 1 Day 1. **Substudy 1 Specific:** CLL transformation to more aggressive lymphoma. Cohort 1B excludes bleeding diathesis, CYP3A inhibitor or inducer use, history of ICH or stroke within 24 weeks, GI malabsorption, and concurrent vitamin K antagonist. **Substudy 3 Specific:** Mediastinal grey-zone lymphoma, Burkitt, Richter's transformation, primary effusion LBCL. Cumulative dose of anthracycline >150 mg/m².

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered via a double step-up dosing schedule in Cycle 1; target dose given every 2 weeks. Combination cohorts receive Acalabrutinib 100 mg orally twice daily (BID) beginning at cycle 2, or R-CHOP administered once every 3 weeks for 6 cycles. **Route of Administration:** AZD0486 via Subcutaneous (SC) injection or Intravenous (IV) infusion. Acalabrutinib (Oral) and R-CHOP (IV/

Oral). **Duration of Treatment:** 24 cycles (28-day) in Substudy 1; 12 cycles (28-day) in Substudy 2; 17 cycles (21-day) in Substudy 3.

5.2 Concomitant & Prohibited Therapy Permitted: Standard of care concomitant therapies necessary for the administration of R-CHOP (e.g., prednisone equivalents). **Prohibited:** Any anticancer therapy within 5 half-lives or 21 days prior to treatment; radiation therapy within 28 days. CYP3A inhibitors/inducers and vitamin K antagonists are specifically prohibited for Substudy 1B.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, LVEF confirmation, CD19 expression check, Baseline Labs
Baseline	Day 0 (Cycle 1)	First Dose (Step-up), Vital Signs, Safety Monitoring
Interim	Treatment Period	Safety Labs, Efficacy Assessment, Dose Escalation Review, Pharmacokinetics
End of Study	Follow-up	Final Vital Signs, Exit Interview, Follow-up for disease progression/survival

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definition: Assessment of safety and tolerability to determine the recommended phase 2 dose (RP2D) for AZD0486 monotherapy and combinations across mature B-cell malignancies. **Measurement Method:** * Number of Participants with Adverse Events (AEs), Serious Adverse Events (SAEs), and Adverse Events of Special Interest (AESIs). * Number of Participants with Dose-Limiting Toxicities (DLTs).

7.2 Secondary Endpoints * Objective Response Rate (ORR) * Complete Response (CR) Rate * Duration of Response (DoR)

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Clinical tracking for immune effector cell-associated neurotoxicity syndrome (ICANS) and cytokine release syndrome (CRS), with specific exclusion for prior Grade ≥ 3 events. **Serious Adverse Events (SAEs):** Standard reporting timelines as defined by the master protocol. **Data Monitoring Committee (DMC):** Internal review committee utilizing a modified probability interval (mTPI-2) design for continuous dose-escalation and safety decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose); Efficacy Evaluable Set. **Sample Size Justification:** 276 patients appropriately powered for dose escalation modeling via mTPI-2 framework across the designated substudies. **Handling of Missing Data:** Standard survival analysis methods with right-censoring for time-to-event efficacy measures (DoR).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Routine onsite and remote monitoring for master protocol adherence. **Audit Trail:** eCRF locking and query resolution via standard clinical data management systems.

11. Study Identifiers & Governance

Primary ID: D7407C00001 **Registry Number:** NCT06564038 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Soundtrack-E **Official Regulatory Title:** A Phase I/II Open-Label Multi-Centre Master Protocol to Evaluate the Safety and Efficacy of AZD0486 Monotherapy or in Combination With Other Anticancer Agents in Participants With Mature B-Cell Malignancies

12. Clinical Framework (Mature B-Cell Malignancies Specific)

Primary Conditions: Chronic Lymphocytic Leukaemia, Small Lymphocytic Lymphoma, Mantle-cell Lymphoma, Large B-cell Lymphoma, B-cell Non-Hodgkin Lymphoma. **Diagnosis Threshold:** Histologically confirmed CD19+ hematological malignancies, Relapsed/Refractory to standard lines of therapy as defined by substudy criteria. **Medical Methodology:** Targeted bispecific T-cell engager (CD19xCD3) therapy. **Optimization Target:** Identifying the optimal safe dosing schedule (double step-up) to minimize CRS and maximize T-cell cytotoxicity.

13. Study Procedures & Methodology

Management Pathway: Step-up dosing administration with specific monitoring periods for CRS/ICANS. **Education Protocol (HCP):** Site training on prompt identification and mitigation of immune-related adverse events. **Education Protocol (Patient):** Education on reporting neurological symptoms or signs of systemic inflammation immediately. **Quality Audit Mechanism:** Continuous safety review required before escalating to the next dose level.

14. Observation & Follow-Up Schedule

Total Duration: Up to 24 cycles of treatment + Follow-up period.
Onsite Visit Cadence: Pre-defined dosing days (e.g., every 2 to 3 weeks depending on the cycle and cohort). **Remote Monitoring Frequency:** Inter-cycle symptom tracking as per investigator discretion. **Checklist Review Interval:** Prior to every infusion/injection to evaluate for unresolved toxicities.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				